

The Child Protection Process A Guide for parents and Carers

Safeguarding Children Team
Information for Patients



Key Principles of Safeguarding Children in University Hospitals of Leicester NHS Trust

- Injuries are common in childhood, but can result in the hospital needing to undertake a detailed discussion in line with our safeguarding procedures to ensure that all children and families are treated equally and that the safety of the child is paramount.
- Staff at University Hospitals of Leicester NHS Trust (“UHL”) will always act in the best interests of the children in our care. A major part of this will be to safeguard their welfare by endeavouring to protect them from neglect and physical, sexual and emotional harm.
- When child protection concerns are raised, the child protection investigation will be carried out according to UHL internal procedures, at the same time as medical investigations to ensure that any medical reason for a child’s presentation is fully considered
- A proper balance must be struck between protecting the child and respecting the rights and needs of parents and families; but where there is conflict the child’s interests must always come first.

Why have I been given this leaflet?

You have been given this leaflet to help you understand what happens within UHL when there is a concern that a child or young person has been or may be being harmed in some way.

If you have been given this leaflet to read, it will probably be because someone is concerned about you and your child.

When any professional receives information that causes them to be concerned that a child may be at risk of, or may have suffered abuse or neglect, they have a duty to share this information with children's social care.

A social worker then has a duty to investigate this matter under Section 47 of the Children Act 1989. UHL staff will work with the social workers, police and any other professionals involved with your family as part of a child protection investigation.

All professionals understand that this process may make you feel worried, upset, confused and perhaps angry because they are looking into an allegation that your child may have been abused or neglected

What happens in UHL when a concern has been raised?

In all but exceptional cases, you will be informed that a referral has been made to social care. You will be informed of the reason for the concern.

All children and young people who are the subject of concerns will be admitted to the Children's Hospital under the care of a Consultant Paediatrician.

In some cases, your child will be admitted under more than one consultant if specialist opinions are required, such as the plastics team for burns, and the orthopaedic team for bone breaks.

Your child will be physically examined by the medical staff and a health history taken from you and, where possible, your child. The doctors will record all this information in your child's notes.

The hospital staff will then speak to a social worker to report the findings of their assessment and a decision will be made as to any further action in the hospital that is required.

What happens if the doctors are not concerned once they have examined my child?

Following examination, the doctor may feel there is no need to take any further medical action in the hospital, and this will be shared with the social worker. The social worker will then decide what follow up, if any, is required from them.

What if further medical investigations are required?

If, after the initial assessment, further information is required, the Consultant Paediatrician will arrange for any medical investigations to take place. The investigations detailed overleaf may be carried out, dependant on the assessment of the medical staff:

Do I have to give permission for these investigations?

All professionals aim to work in partnership with families. In most cases, at each stage of the investigation and for each medical investigation, we will ask for your permission. However, if you decide not to give permission and it is deemed in your child's best interests to gather the medical information, permission for the investigations will be given by the Paediatric Consultant.

What medical investigations might you do?

CT Head:

A CT head scan allows the doctors to see inside the head to check for any injuries or problems. This is carried out on children up to two years old where there are child protection concerns of physical injuries.

Ophthalmology Review:

This is an examination of your child's eyes by a specialist doctor. This allows them to look into your child's eyes and see if there are any injuries or problems.

Blood Tests:

These may be taken on any child to show any medical problems that can show up in the blood, including infections or a problem with clotting which can then cause excessive bleeding and bruising.

Photographs:

If there are physical injuries or marks to your child, the medical staff will request the hospital Medical Illustration Team or Police Scenes Officers take photographs of the marks. This provides a record for the hospital and other professionals of the marks.

Skeletal Survey:

This is carried out on children up to two years old where there are child protection concerns of physical injuries. The skeletal survey is a number of x-rays of the whole of your child's body, which can show up any bone breaks or abnormalities. It will also show up any old breaks on bones.

After any skeletal survey, a chest x-ray will be carried out approximately two weeks later. This is because some rib fractures can take time to show up, and so the repeat x-ray checks for these.

Does a skeletal survey or bone scan lead to an excessive radiation dose for the child?

Any x-ray carries a radiation dose, and this must be balanced against the potential gain from performing the x-ray. A skeletal survey varies in the amount of radiation, but is roughly equivalent to the radiation from 7-20 transatlantic flights. A bone scan is higher than this but the risk from radiation is still regarded as low. The risk of harm from the x-ray is far outweighed by the benefit of detecting fractures in an infant less than two years of age and protecting them from further abuse.¹

1. NSPCC Core-Info: Fractures in Children found at <http://www.nspcc.org.uk/globalassets/documents/advice-and-info/core-info-fractures-children.pdf> (accessed 28th July 2015)

What happens next.....

Hospital staff will contact your child's Health Visitor or School Nurse and your child's GP to make them aware of the investigation and to gather information they know about you.

If your child is under 6 months old, the child's mother's maternity notes will be reviewed by the Safeguarding Midwife. This ensures that any information relating to your antenatal period or the delivery of your child is known as this may guide medical investigations.

You are likely to be visited on the ward by the Social Worker and Specialist Police Officers from the Child Abuse Investigation Unit. They will talk to you about the concerns that have been raised. Their job is to find out what has happened and to decide whether your child is at risk of, or has been harmed.

The Social Worker and Police Officer will ask you, and sometimes other family members and friends, a lot of questions. You will have the opportunity to give your point of view and ask any questions you may have.

It will always be necessary for your child to be seen by the Social Worker, as it is your child's welfare which is the main concern. If your child is old enough, they will also be spoken to by the Social Worker and Police Officer.

In all children up to 2 years, the UHL Safeguarding Team (made up of nursing staff) will call a Safeguarding Information Sharing Meeting while your child is still in hospital. This is for the hospital staff to meet with the Social Worker and Police Officer to share the results of all the medical investigations and to give any opinions and outline any further investigations that may be required. The Social Worker and Police Officer can then ask any questions they may have about the medical investigations. This meeting provides further information to the Social Worker for their child protection investigation to ensure the safety of your child.

These meetings may also be held for children over 2 years where it is deemed appropriate.

Parents are not invited to this meeting, but will be informed of the key information immediately after the meeting. This is usually by the Social Worker.

What happens when the hospital investigations are complete?

The hospital staff will be guided by the Social Worker on their continuing involvement with you and your child. UHL cannot discharge your child until the Social Worker is satisfied that your child will be safe on discharge. The Social Worker will discuss discharge arrangements with you.

Once UHL are advised of the discharge and follow-up arrangements by the Social Worker, and your child is medically well enough to leave the hospital, your child will be discharged.

Your child's GP and Health Visitor or School Nurse will be notified of the outcomes of all medical investigations and of your discharge arrangements.

The Social Worker will advise you of how they plan to take your case forward. This may be for no further action, some family support, or the need for a child protection conference. Whichever course is followed, the Social Worker will explain this to you.

What if I am not happy about the Child Protection Investigations at UHL?

UHL staff aim to provide you and your child with high quality care. Staff will be open and honest with you about the processes that are being followed. If you are unhappy about any aspect of your care at UHL, please discuss these with your ward nurse, Consultant Paediatrician, or ask to see the Children's Hospital Matron.

If your complaint is regarding the investigation as a whole, please discuss this with the Social Worker who has seen you at the hospital. If you prefer, you can ask to talk to the Social Worker's Team Manager.

If there is anything you do not understand, please ask:

We understand this will be a very difficult time for you and your family. These are however procedures that must be undertaken by law to ensure that a consistent approach is taken to ensuring the children and families in our care are safe.

It is normal for you and your family to have many questions about this process, and our staff will try to answer them for you. If they do not have the answers for you, they will try to find someone who does.

Questions

If you have any questions, write them down here to remind you what to ask when you speak to your nurse/consultant.

[illegible]

[illegible]

[illegible]

Useful Contact Numbers

Leicester City Children's Social Care (Greyfriars):

0116 454 1004 (24hours)

Leicestershire County Children's Social Care:

0116 305 0005

(24 hours)

Rutland Children's Social Care:

01572 758 407 (Office hours only)

(outside of office hours contact Leicestershire County Children's social care)



Today's research is tomorrow's care

We all benefit from research. Leicester's Hospitals is a research active Trust so you may find that research is happening when you visit the hospital or your clinic.

If you are interested in finding out how you can become involved in a clinical trial or to find out more about taking part in research, please speak to your clinician or GP.

If you would like this information in another language or format, please contact the service equality manager on 0116 250 2959

إذا كنت ترغب في الحصول على هذه المعلومات في شكل أو لغة أخرى ، يرجى الاتصال مع مدير الخدمة للمساواة في 0116 250 2959.

আপনি যদি এই লিফলেটের অনুবাদ - লিখিত বা অডিও টেপ এ চান, তাহলে অনুগ্রহ করে সার্ভিস ইকুয়ালিটি ম্যানেজার ডেভ বেকার'এর সাথে 0116 250 2959 নাম্বারে যোগাযোগ করুন।

如果您想用另一种语言或格式来显示本资讯，请致电 0116 250 2959 联系“服务平等化经理” (Service Equality Manager)。

જો તમને આ પત્રછાનું લેખિત અથવા ટેપ ઉપર ભાષાંતર જોઈતું હોય તો મહેરબાની કરી સર્વિસ ઇક્વાલિટી મેનેજરનો 0116 250 2959 ઉપર સંપર્ક કરો.

यदि आप को इस लीफलेट का लिखती या टेप पर अनुवाद चाहिए तो कृपया डेव बेकर, सर्विस ईक्वालिटी मैनेजर से 0116 250 2959 पर सम्पर्क कीजिए।

Jeżeli chcieliby Państwo otrzymać niniejsze informacje w tłumaczeniu na inny język lub w innym formacie, prosimy skontaktować się z Menedżerem ds. równości w dostępie do usług (Service Equality Manager) pod numerem telefonu 0116 250 2959.

ਜੇਕਰ ਤੁਹਾਨੂੰ ਇਸ ਲੀਫਲੈਟ ਦਾ ਲਿਖਤੀ ਜਾਂ ਟੇਪ ਕੀਤਾ ਅਨੁਵਾਦ ਚਾਹੀਦਾ ਹੋਵੇ ਤਾਂ ਕਿਰਪਾ ਕਰਕੇ ਡੇਵ ਬੇਕਰ, ਸਰਵਿਸ ਇਕੁਅਲਿਟੀ ਮੈਨੇਜਰ ਨਾਲ 0116 250 2959 'ਤੇ ਸੰਪਰਕ ਕਰੋ।

Ak by ste chceli dostať túto informáciu v inom jazyku, alebo formáte, kontaktujte prosím manažera rovnosti služieb na tel. číslo 0116 250 2959.

Haddaad rabto warqadan oo turjuman oo ku duuban cajalad ama qoraal ah fadlan la xiriir, Maamulaha Adeegga Sinaanta 0116 250 2959.

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